جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : رحيل سعد سدراك ميخائيل
المحافظة : القاهره **الحي / المدينة :** البساتين **العنوان :** دار السلام
رقم البطاقة : 25206262700161 **رقم التليفون :** 01281040605 **تاريخ تسجيل الإستمارة :** 20-10-2026
العمر(تاريخ الميلاد) : 26-06-1952 **المهنة :** بدون عمل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: رحيل سعد سدراك ميخائيل
National ID: 25206262700161

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Bilateral mammogram and breast ultrasound**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين

ERM036745948





National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:
1- Name : رحيل سعد سدراك ميخائيل
Sex: Female
3- Medical number: 83252166
Old Med : 97-2018
2-ID number:
Patient Age : 73 Y

MDT

Family history	-ve	Phone	01281040605	BMI		REASON OF MDT	
marital status		NRS Score		GROUP		Date of MDT	01/02/2025
Co morbidity	dm	Menopausal status		Initial STAGE			

Clinical data

Breast Imaging :
rt breast mass at 10 o'clock 2.4x1.3 6 cm from nipple with non specific LN BIRADS 4c

Mets Workup :
cap: free
u/s:heaptomegaly with rt renal simple cyst

BX	ER	PR	Her 2 Neu	SISH	Ki67
IDC	8/8	5/8	-ve		

T	N	M

Sonomammography

PATIENT HISTORY
PT RECIVED CHEMO SINCE 2017 FOR ENDOMITRAL CARCINOMA
RT UPFRONT BCS+SLNB: free margin
LN 1/4

MDT OUTCOME & PLAN

FOR BILATRAL LOWER LIMP DUPLEX
2- MRI PELVIS (SEVER PELVIC PAIN)
IF NO METS FOR 12 WKS AXOL+ ENDOXAN d1, PORT
AI X 5 YRS ZOMETA Q 6 MONTH AFTER DENTAL

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY	RADIOLOGY	RADIOLOGY
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric		Nutrition	

Patient Name	: الأستاذه / رحيل سعد سدراك مخائيل	Request No.	: 13004756
Patient No.	: 25206262700161	Request Date	: 06/11/2024
Path No.	: 4294/24	Verification Date	: 13/11/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 72 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Right breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 5/11/2024

Gross Examination:
- 4 grayish white tissue cores up to 1.2 cm, totally submitted.

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
* Ductal carcinoma in situ: Not detected
* Perineural invasion: Not detected
* Angioinvasion:Not detected.

Diagnosis:
Right breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade II.

Signature
Prof. Dr. Lubna Omar
Dr. Menna Nabil

Verified by :
DR.Menna Nabil

غرفة رقم (4) – الدور الأرل – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qudema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / رحيل سعد سدراك مخائيل	Request No.	: 13000306
Patient No.	: 25206262700161	Request Date	: 31/12/2024
Path No.	: 5280/24	Verification Date	: 15/01/2025
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 72 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Right breast cancer, Up-Front..
Nature Specimen: Right breast conservative surgery.

Gross Examination:
* Procedure: Right BCS
Fibrofatty tissue piece measured 9x9x3 cm with no covering skin.
* Tumor extent: An ill-defined greyish white firm mass:
* Size: 2.5x2x1.5 cm.
* Distance from margins: 0.4cm away from deep margin.
*SNLs:FFT piece measured 5x5 cm entangling 4 nodules up to 0.6 cm.

Microscopic Examination:
* Histological type:Invasive duct carcinoma with focal DCIS
* Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade II (3+2+1)
* Ductal carcinoma in situ: Focal
* Resection margins: Free
* Microcalcifications: Nott detected
* SNLs: Positive metastatic deposits in 1 nodule out of 4 (1/4)

Diagnosis:
Right BCS, Up-front:
Invasive duct carcinoma with focal DCIS ,Grade II
- Free margins
- SNLs: Positive metastatic deposits in 1 nodule out of 4 (1/4)
- Stage: pT2, pN1

Signature

Verified by :
Dr.Heba Hasan

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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / رحيل سعد سدراك مخائيل	Request No.	: 13004756
Patient No.	: 25206262700161	Request Date	: 06/11/2024
Path No.	: 4294/24	Verification Date	: 26/11/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 72 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast mass.

Nature of Specimen: Tru-cut biopsy,collected at 5/11/2024.

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:
* IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 90% of tumor cells (score 8 /8).
- PR: Moderate nuclear reaction in 25% of tumor cells(score 5 /8).
- Her2: No membrane reaction in the tumor cells (score 0).
- Ki67: 25% nuclear reaction of tumor cells.

Conclusion:
Right breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade II.
IPT:
- ER: Positive (score 8/8).
- PR: Positive (score 5/8).
- Her2:Negative (score 0).
- Ki67: Positive in 25% of tumor cells.

* Scoring system: "ER, PR"

Verified by :

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	الأستاذه / رحيل سعد سدراك مخائيل	Request No.	13004756
Patient No.	25206262700161	Request Date	06/11/2024
Path No.	4294/24	Verification Date	26/11/2024
Sex	Female	Financial Category	اورام دار السلام - نفقة الدولة
Age	72 Y	Department	Out Patient
Referreing Doc.		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Prof. Dr. Lubna Omar
Dr. Menna Nabil

Verified by :

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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

PATHOLOGY REPORT

Name : Raheel Said Date : Sept 2017 Code : G-9-6	الاسم : رحيل سعيد التاريخ: الاستاذ الدكتور : هشام جابر العناني

Gross :
Curetting; 2x2, totally embedded.

Microscopic:
Sections examined revealed fragments of endometrial tissue showing neoplastic growth formed of villoglandular structures, lined by neoplastic cubical cells with mild to moderate anaplasia.

Diagnosis :
ENDOMETRIAL BIOPSY:
LOW GRADE, VILLO-GLANDULAR TYPE,
OF ENDOMETRIOID ENDOMETRIAL ADENOCARCINOMA.

HANY KHATTAB, M.D.
Professor of Pathology

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

CAIRO UNIVERSITY
FACULTY OF MEDICINE
PATHOLOGY DEPARTMENT

Pathology No: 1697n Age: 65y Sex: female

Name: [illegible] Hospital no:

Clinical diagnosis: Endometrial cancer

Nature of specimen: Panhysterectomy

Department: [illegible] Date: 09/11/2017

PATHOLOGY REPORT

Gross
Total hysterectomy specimen. Cervix, 3 cm long & uterus, 6x5x4cm. Endometrium showed a friable whitish-tan growth, filled the cavity, invading more than half the myometrium (1.7 cm thick) & reaching 0.5 cm from the internal os. Each ovary was 2x2x1 cm & each tubes was 2 cm long.

MICROSCOPIC
Examination of the specimen received revealed sections in uterine wall showing infiltration by malignant growth formed of papillae lined by highly pleomorphic tumor cells containing prominent nucleoli. The growth was infiltrating more than half of myometrium.
The cervix showed chronic non-specific endocervicitis.
The right ovary showed thecoma fibrosis.
The left ovary & both tubes were unremarkable.
Both parametria were free of tumor deposits.

DIAGNOSIS
Pan hysterectomy:
Papillary serous carcinoma, high grade, infiltrating more than half the myometrium. Free cervix, free ovary and free parametria.

PROF.DR. AHMED ABO EL-AZIZ
PROF.DR. DINA OMAR

[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY INTERNATIONAL EGYPT

Patient Name: Rahil Saad Sedrak Mekhail	Date : 31/08/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 73 Year(s)
Medical ID: 97/2018	Pat. ID: 16979

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

I- CHEST:

- Few right axillary lymph nodes largest measuring 14 mm in diameter.
- Right lower lobe subpleural pulmonary nodule measuring 4.6 mm.
- Right basal fine atelectatic bands.
- No pulmonary masses or cavitary lesions.
- No consolidation or collapse.
- Few pretracheal and prevascular lymph nodes largest measuring 19 mm at the pretracheal region.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.
- Bilateral fibro-elastoma dorsi.

II- ABDOMEN AND PELVIS:

- Status post hysterectomy showing clear operative bed.
- Average sized liver showing prominent left hepatic lobe and caudate lobe with deep interlobar fissure denoting chronic parenchymatous liver disease for lab assessment. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes. Two right renal cortical cysts.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Left pelvic small calcific focus is noted.
- Normal appearance of the rest of pelvic organs with no pelvic masses or collections.
- Multiple subcentimeteric bilateral iliac and para-aortic lymph nodes.
- No ascites.
- No worrisome lytic or sclerotic bone lesions.
- For PET-CT assessment.

Dr. Sherif Wagdy

CT Scan
[[DOCTYPE:CT_SCAN]]

Name Raheel Saad Sedrak

Date : 26/12/2021

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

A known case of endometrial cancer post-therapeutic follow up.

1. ***Chest:***

- Clear both lung fields
- No sizable pulmonary masses, areas of consolidation or cavitary lesions.
- Prominent mediastinal lymph nodes are seen, largest measures 1.6x1.2cm.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Bilateral elastofibroma dorsi.
- Normal appearance of the dorsal vertebrae.

2. ***Abdomen and pelvis:***

- Evidence of TAH&BSO. No evidence of operative bed masses or collections.
- The liver is of enlarged size with homogenous low CT texture(fatty liver). No focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No abdominal or pelvic lymphadenopathy or collections.
- Diffuse osteopenic texture with spondylo-degenerative changes yet no lytic or sclerotic bone lesions.

OPINION:

- ***A known case of endometrial cancer post-therapeutic follow up, showing no evidence of local recurrence or metastatic disease.***

DR/Ibrahim Kamel, FEBR-ABR

Dr. Ola Magdy, MD

CT Scan
[[DOCTYPE:CT_SCAN]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــحة والسكـان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Rahil Saad Sedrak Mekhail	Date : 02/12/2024
Exam : C.T CHEST	Age : 72 Year(s)
Medical ID: 97/2018	Pat. ID: 16979

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS
REVEALED: -

- **A known case of endometrial cancer post-therapeutic follow up.**
- **Compared to the previous study dated 4-11-2024.**

1. **Chest:**
   - No sizable pulmonary masses, areas of consolidation or cavitary lesions.
   - Still seen right lower lung lobe apical segment subpleural small nodule measures about 5 mm.
   - Still seen prominent mediastinal lymph nodes are seen, largest measures 2.1 x 1.6 cm.
   - No evidence of pleural effusion, pleural thickening or masses seen.
   - Normal cardiac size. No pericardial effusion detected.
   - Bilateral elastofibroma dorsi are still seen.
2. **Abdomen and pelvis:**
   - Evidence of TAH-BSO with still no evidence of operative bed masses or collections on non-contrast basis.
   - The liver is of enlarged size with homogenous low CT texture (fatty liver). No focal lesions on non-contrast basis or biliary radicles dilatation.
   - Normal size & parenchymal texture of the spleen with no focal lesions seen.
   - Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes. Right renal cortical cysts.
   - Prominent porto-caval lymph node is still noted.
   - Normal mc CT appearance of the pancreas and both suprarenals.
   - Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
   - No abdominal or pelvic collections.
   - No destructive lytic or sclerotic bone lesions.

OPINION:

- **Follow up status showing still no evidence of operative bed masses or distant metastases on non-contrast basis.**

Dr. Ali Elsisy, MSc.

Prof. Menna Tallah Hanafy, M.D.

CT Scan
[[DOCTYPE:CT_SCAN]]

Ministry Of Health
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Radiology Department

وزارة الصحة والسكان
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مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Rahil Saad Sedrak Mekhail	Date : 04/11/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 72 Year(s)
Medical ID: 97/2018	Pat. ID: 16979

Combined bilateral breast mammography and complementary ultrasound

Indication:

right breast palpable lump.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- right breast shows UOQ spiculated irregular mass lesion, of high density.
- Complementary ultrasound revealed right breast irregular spiculated hypoechoic mass lesion, with subtle vascularity detected within on color Doppler, seen at 10 o'clock position , sone C(6cm from the nipple, 0.5cm from the overlying skin, measuring 2.4x1.3 cm, consistent to the palpable lump,
- Normal skin thickness, and nipple-areola complex of the left breast.
- Left axillary single LN showing mild cortical thickness reaching 4mm
- Nonspecific axillary LNs.
- Free right supraclavicular and infraclavicular regions.

Opinion:

- Right breast 11 o'clock UOQ malignant looking mass lesion (BIRADS 4c) for histopathological correlation.
- Benign findings left breast (BIRADS 2)
- Left axillary single LN showing mild cortical thickness for follow up.

Yours truly

Signature Dr. basma mohmaed

04/11/2024 2:22 PM

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Rahil Saad Sedrak Mekhail	Date : 04/11/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 72 Year(s)
Medical ID: 97/2018	Pat. ID: 16979

Combined bilateral breast mammography and complementary ultrasound

Indication:

right breast palpable lump.

Technique

- Bilateral MLO and CC views
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Report:

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Opinion:

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- Benign findings left breast (BIRADS 2)
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Yours truly

Signature Dr. basma mohmaed

04/11/2024 2:22 PM

Mammogram
[[DOCTYPE:MAMMOGRAM]]